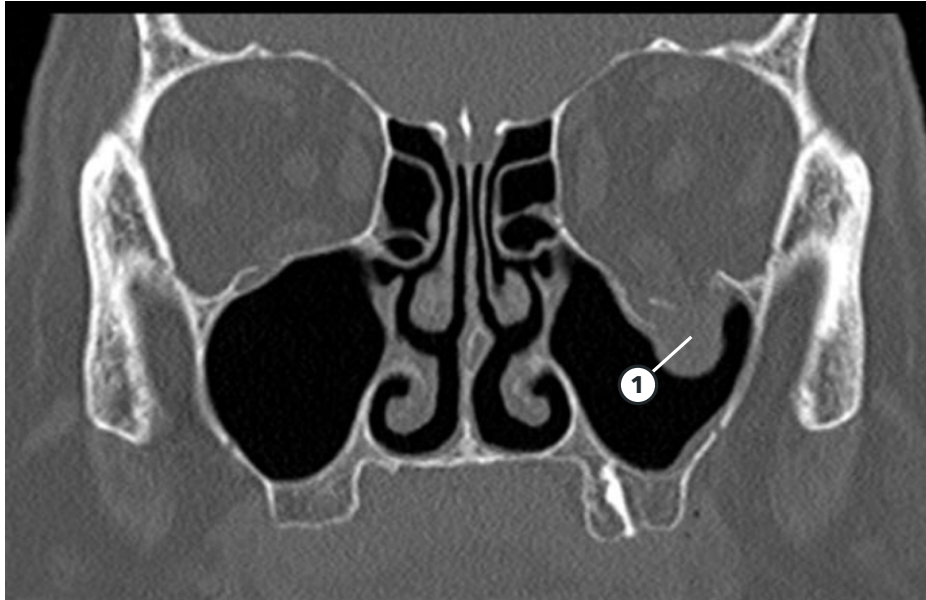


The white eye orbital fracture

CLINICAL M3-M5 | TYPE B | A focused decision guide

Symptoms can outweigh bruising

An orbital floor defect



Illustrative supplied coronal CT: floor disruption with soft tissue extending into the maxillary sinus on the image right. The scan alone does not prove muscle entrapment. Supplied M3 Anki, media 7.

Look for the mismatch

Clue	Why it matters
Little bruising but painful restricted elevation	A child may have a trapdoor fracture with entrapped tissue.
Nausea, vomiting or bradycardia with eye movement	Possible oculocardiac reflex.
Preserved acuity	Does not exclude muscle entrapment or its systemic effects.

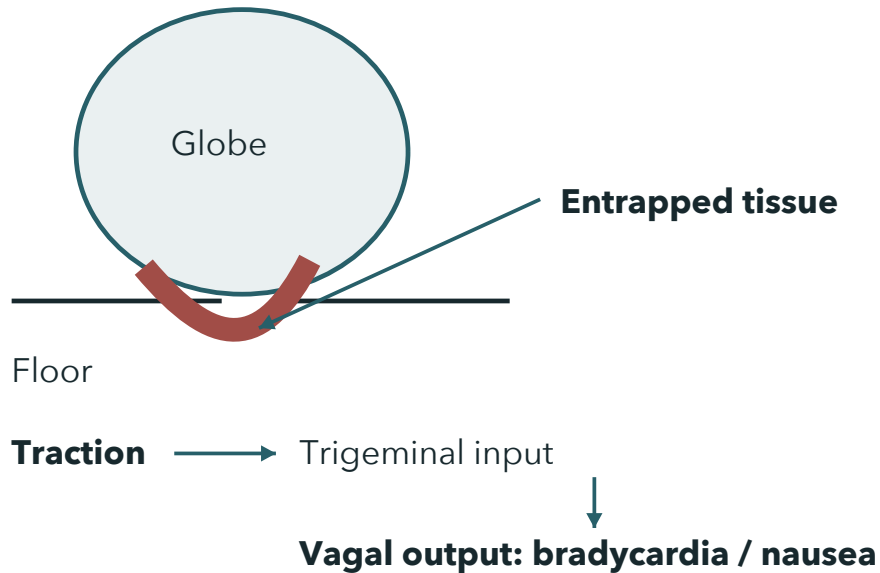
Read the CT with the examination

A subtle fracture does not neutralise severe clinical restriction. Entrapment is a clinical and imaging judgement.

References AAO: Orbital floor fractures | AAO: Oculocardiac reflex

Separate urgent release from observation

An eye movement can trigger vagal symptoms



Original pathway schematic. Ocular traction activates a trigeminal afferent and vagal efferent reflex.

Choose the appropriate pathway

Clinical situation	Response
Marked restriction with vagal symptoms	Urgent orbital review, cardiac monitoring and release assessment.
Improving diplopia without entrapment or instability	Specialist observation may be appropriate.
Persistent disabling diplopia or troublesome enophthalmos	Reassess for elective repair.

The timing trap

Do not apply a routine delayed-repair schedule to a child with an oculocardiac response. Conversely, not every uncomplicated adult fracture needs immediate surgery.